

Sarah Levy, MD — Internal Medicine

1492 Bryant St, Suite 305 — San Francisco, CA 94110

Patient: Chen, Sarah **DOB:** 11-Jun-1976 **MRN:** 4471028
Date of Service: May 10, 2023 **Visit type:** Diabetes follow-up

HPI

Pt returns for f/u of T2D. A1c 8.4% on metformin 500 b.i.d. Tolerating well, no GI sx.

Sarah returns for a 3-month follow-up after starting metformin in January. She reports good adherence; she takes it with breakfast and dinner without fail. No GI side effects, no bloating, no diarrhea. Her energy has improved noticeably and the polydipsia has resolved. Home BG monitoring shows fasting values trending in the 130–150 range, down from initial 180s.

April lab work (drawn 04/18/2023) showed A1c improved from 9.2% to 8.4%, which is encouraging but still above the goal of <7.0%.

REVIEW OF SYSTEMS

Constitutional: improved energy. GI: no complaints. GU: no nocturia. Skin: no new lesions.
Endocrine: no symptoms suggesting hypoglycemia.

VITALS

BP 128/80 HR 74 T 98.2°F Wt 174 lbs (down 4 lbs from January).

EXAM

General: well. CV/Pulm/Abd: unremarkable. Feet: monofilament intact bilaterally, DP and PT pulses 2+, no ulcerations.

ASSESSMENT & PLAN

1. **T2D, sub-optimal control.** A1c 8.4%, response to metformin partial. Will increase dose. Reinforced importance of consistent meal timing.

Δ metformin → 1000 mg PO b.i.d. F/u A1c in 3 months.

Patient counseled to titrate over 5 days (one extra 500 mg tab with dinner for 5 days, then add the morning increase) to minimize GI upset.

2. **Lifestyle.** Encouraged to continue current dietary changes. She has started walking 20 min after dinner most evenings. Continue.

3. **Lipids** — already noted borderline 01/2023, defer to 6-month recheck.

Counseling: 15 min on titration plan, hypoglycemia awareness, and what to do if she misses a dose.

Next appointment: 08/10/2023 at 1015 with Dr. Levy.

Lab order placed: repeat HbA1c, drawn 1 week before visit.

— Sarah Levy, MD